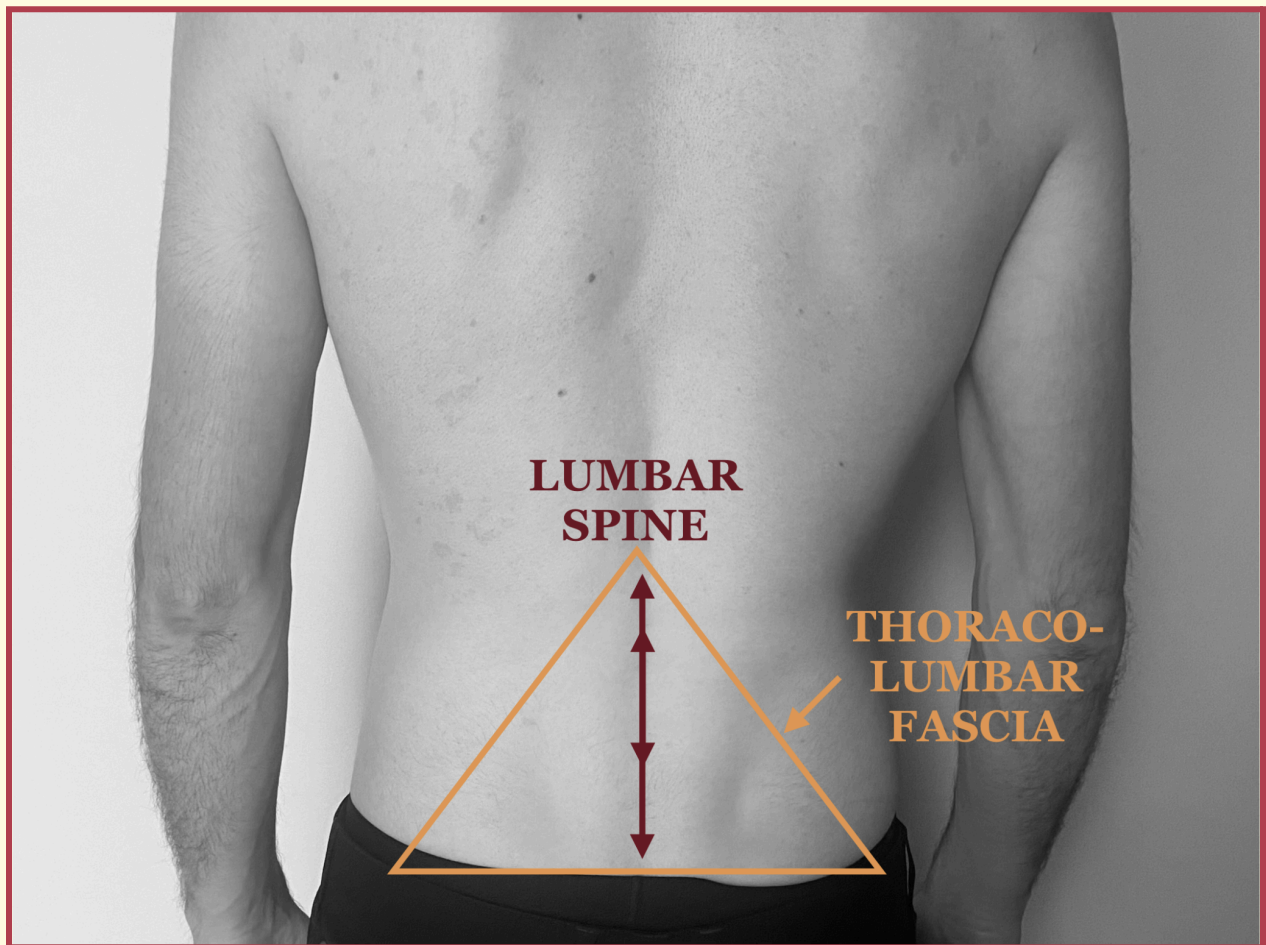


LOW BACK PAIN

Can Massage Therapy Treat Pain In The Lower Back?



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Massage therapy is a common and effective treatment for low back pain. It reduces pain, eases muscle guarding, and improves comfort, relaxation, and sleep, particularly in the short term. Regular physical activity, strength training, and adequate rest are also necessary in treating the problem as something more systemic than a single tight knot or misaligned joint.

WHAT IS LOW BACK PAIN?

Low back pain is one of the most common reasons people seek out massage, and *it is one of the most common health complaints* in general. Roughly 70 to 80 percent of adults will experience it at some point in their lives, and a substantial portion of those people go on to develop chronic, recurring pain rather than a single isolated episode.

Most low back pain is what clinicians call "**non-specific**," meaning *there is no single, clearly identifiable structure that can be blamed as the source*. It typically presents as an ache, stiffness, or deep soreness across the lumbar region, sometimes radiating into the hips or glutes, and often accompanied by reduced range of motion. It may flare after a specific event, such as an awkward lift, but frequently it appears without any obvious cause. **Acute** low back pain tends to resolve on its own eventually. The more stubborn version is the **chronic** form that lingers or repeatedly returns.¹

A small number of symptoms are red flags for something requiring medical attention rather than bodywork, such as pain accompanied by numbness or weakness in the legs, loss of bladder or bowel control, unexplained weight loss, or fever. Any of these warrants a physician rather than a massage table.

Anatomy

Specific back pain injuries like **herniated discs** and **pulled muscles** can be attributed to specific anatomical components and addressed locally, but non-specific low back pain is by definition not caused by any specific injury or disorder and requires variable client-specific treatment broadly across the system.

Spanning, crossing, and articulating the lower back and hips are several powerful and complicated muscles so *a plethora of minor dysfunctions can contribute to systemic regional pain*. The **thoracolumbar aponeurosis**, **erector spinae aponeurosis**, and other fascial components build a wide sheet of highly-innervated connective tissue in the lumbar region with complex connections to crucial postural muscles involving sections of fascia which should glide against one another and sections which should anchor to one another.²

Pathology

In clinical studies, clients suffering from low back pain have been proven to have reduced gliding in the aforementioned fascial layers of the lumbar region. This is a strong association, but not necessarily a clinically proven causation in either direction. Nonetheless, there is a growing evidence-base for the theory that the **hyaluronic acid** which separates and lubricates the sheets to assist with gliding becomes more dense and concentrated. Injuries may also stimulate **fibroblasts** to produce additional **collagen fibers** producing tangles and knots in the sheets.²

It bears reinforcing that *no single cause can be attributed to non-specific low back pain*. These changes in the fascia may lead to pain which leads to **muscle guarding** which leads to further dysfunction. **Anterior pelvic tilt** and general pelvic misalignment is a common association in non-specific low back pain, particularly in office workers, and regularly presents with hip-rotation and trunk flexion/extension limitations, sometimes to the point of disability.⁶

HOW IS LOW BACK PAIN TREATED?

Most *low back pain responds very well to conservative, non-surgical care*, and current thinking has replaced an emphasis on bed rest with one on staying active. Gentle movement, appropriate strength training, the avoidance of prolonged rest, heat, over-the-counter anti-inflammatory medication where appropriate, and hands-on care all have a place.

Massage Therapy

Massage therapy is an important part of improving both pain and function in people with low back pain. Given that non-specific low back pain so often involves the fascial and muscular dysfunctions described above, manual therapy for low back pain must focus on those areas. **Myofascial connective tissue massage**, which works specifically on the superficial fascial layers, and more conventional classical massage have both been shown to reduce pain in clients with chronic mechanical low back pain, alongside measurable improvements in disability, sleep quality, and autonomic markers such as heart rate and blood pressure.³

There is also emerging evidence that manual and mechanical techniques can act on the thoracolumbar fascia in the way predicted by the fascial gliding dysfunction model of low back pain pathology described above. **Percussive massage** applied to the low back has demonstrated improvement in the viscosity of the hyaluronic acid that governs how the fascial layers glide. Proper sports massage techniques when thoughtfully and regularly applied provide measurable relief of non-specific low back pain.⁵

Effective massage therapy addresses:

- the erector spinae and deep paraspinals
- the quadratus lumborum
- the glutes and hip rotators
- the thoracolumbar fascia

to improve health and support recovery by:

- reducing pain
- easing muscle guarding
- improving comfort and ease of movement
- calming the nervous system
- making it easier to stay active

Behavioral Corrections

Work at home in between massage therapy sessions is a necessary part of long-term treatment. Because non-specific low back pain rarely traces to a single cause, this work must be tailored to the individual rather than pulled from a fixed template, but one essential factor is always to keep moving. Prolonged immobility encourages the same fascial densification and loss of gliding that contributes to the pain, whereas regular healthy movement helps maintain the hydration and mobility of the connective tissue.⁵

Many people with persistent low back pain, especially those who sit for long hours, present with weak or poorly coordinated trunk, hip, and gluteal muscles and with postural dysfunctions such as anterior pelvic tilt. Gradually building strength in these powerful muscles and sharpening postural awareness rebalances the load that is exacerbating the issue. Because the presentation varies from one case to the next, and because over training certain back-muscles is often associated with low back pain, this too must be tailored to the individual.

For most people dealing with low back pain, the priorities between sessions are:

- maintaining low-stress activity such as walking
- avoiding extended stretches held in one position
- using heat and medication sparingly to manage flare-ups
- building strength gradually and safely

WHAT CAN ATLAS BODYWORK DO?

Atlas Bodywork's approach to low back pain centers on helping clients feel better and move more freely. My work focuses on myofascial release of the thoracolumbar fascia and sports massage of the muscles and soft tissue of the low back, hips, and glutes. Sessions also include honest tailored guidance on activity and pacing, to extend the short-term relief gained on the table into something lasting.

References

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